CALIFORNIA ASSOCIATION OF PUBLIC HEALTH ECONOMISTS

Defining Our Path Together

What we shared, what we can hold, and what we choose to build next.

Member Working Session: Third Gathering

OUR SHARED DISCIPLINE

More Than Causal Inference: A Way to Make Choices

Most of us shared that health economics is too often understood narrowly as econometric testing or formal cost-effectiveness analysis. We see it as a fundamental discipline for public health decision-making:

Trade-Offs and Opportunity Costs

Recognizing what we give up when resources are dedicated to one program over another, and tracing out unintended spillovers.

Decision Architecture

Helping public health leadership structure difficult choices before policies are locked, rather than just evaluating them years later.

Deploying Resources to Best Uses

Connecting economic evidence directly to county budgeting, state priorities, and sustainable public health investments.

FOUNDING SURVEY FINDINGS

Where We Stand Together: Clear Common Ground

100% Agreement on the Core Need

All of us identified the lack of a statewide peer network, shared learning, and professional community as our most urgent problem.

Clear Consensus on Primary Mission

Most of us selected increasing institutional demand: helping leaders understand how economics improves decisions, accountability, and the case for prevention.

The Audience We Hope to Influence

Most of us noted that our work must reach local health department senior executives and state public health leadership.

CAPACITY CHECK

Aligning Our Ambition with What We Can Hold

What We Hoped to Build (Ideal Roles)

- Comprehensive toolkit development from scratch
- Statewide formal technical assistance programs
- Direct organizational consulting to local agencies

What We Can Credibly Sustain as Volunteers

- A strong, collegial Community of Practice for each other
- Serving as Translators: turning economic tools into plain stories
- Zero of us felt building heavy toolkits alone was sustainable

Key Takeaway: We do not need to become a massive consultancy. We can be an applied translation engine.

OUR PRODUCTIVE TENSION

Navigating the Two Risks We Identified

In our open responses, members named two real and opposing failure modes for groups like ours:

Risk 1: The Inward Bubble

"We live in a bubble, agreeing that health economics can be an important contributor, but with no strategy or implementation to make it happen."

"We have great discussions but do not improve the lives of our members outside of those discussions."

Risk 2: Premature Overextension

"Limited ability to influence adoption by state and local agencies due to not having the right contacts or visible presence."

"We lose our focus on academic rigor before our foundation is solid."

Our Central Question: How do we build an inward home that powers external translation without burning out?

OUR 18-MONTH HORIZON

What Success Looks Like for Us

Most of us coalesced around three concrete milestones that would define success at 18 months:

1. Reliable Peer Network

Members have an active, trusted peer circle for ongoing consultation, methodology sounding boards, and genuine collaboration across California.

2. Leadership Fluency

Public health directors and agency leaders can clearly describe how health economics supports their budgeting and resource choices.

3. Two to Three Visible Examples

Our group has produced 2 to 3 practical case examples demonstrating how economic framing changed a real California public health decision.

STRATEGIC CHOICES

Where Do We Take the Group Next?

PRIMARILY INTERNAL

Path A: Methods Sanctuary

Core Focus:
Focus on peer review of member manuscripts, causal inference labs, and econometric career support.

Trade-off & Feasibility:
Low volunteer load, but risks becoming the isolated academic bubble.

RECOMMENDED BALANCE

Path B: Translation Engine

Core Focus:
Use internal meetings to workshop practical California dilemmas and co-produce 2-page decision briefs for leaders.

Trade-off & Feasibility:
Sustainable volunteer load, directly addresses institutional demand.

PRIMARILY EXTERNAL

Path C: Executive Advisory

Core Focus:
Offer structured technical assistance to CDPH, write policy memos, and lobby for formal accreditation roles.

Trade-off & Feasibility:
High institutional visibility, but very high risk of volunteer burnout.

OUR OFFERINGS MENU

What Kind of Events and Resources We Can Produce

Events We Host

Monthly Peer Reviews & Translation Clinics
Alternating between peer methodology review and interactive clinics where members workshop live county decision dilemmas.

Applied Webinars on Resource Allocation
Focused external sessions (e.g. Tim Brown on ROI, opportunity costs in local budgets) aimed at program directors and analysts.

No Extra Homework
Using our existing meeting time to co-develop ideas rather than expecting unpaid hours between calls.

Resources We Produce

Two-Page California Decision Briefs
Short, practical case stories showing how economic framing shifted real choices (e.g. food security, clinic allocation, CHWs).

The 'Economic Lens' One-Pager
A five-question checklist for health directors to assess trade-offs and opportunity costs before approving programs.

Curated California Methods Repositories
Disseminating existing datasets, state code, and relevant tools rather than building platforms from scratch.

OUR CONVERSATION TODAY

Choosing Our Next Step Together

Questions for Our Room Today

1. What Resonates?

Does Path B (The Applied Translation Engine) feel like the right balance between our internal community and external demand?

2. What Is Our First Case?

What is one real program dilemma from your current work that we could workshop together in our next session?

3. What Is One Deliverable You Need Most?

If CAPHE produces one practical resource this year, what would help your conversations with directors and partners the most?

Thank you for shaping what CAPHE is becoming. Next session: Our First Applied Translation Clinic.